

LABORATORY INVESTIGATION REPORT

Patient: [REDACTED]
UHID: AHDW.325041
Episode: IP
Ref. Doctor: Dr. Gastro Unit (Dr. Sharad / Dr. Ekant)
IP NO: 127101
Facility: Aakash Healthcare De hi

Biochemistry

Name	Result	Unit	Ref. Range
Sample No. [REDACTED]	Collection Date: 26/03/25 04:54	Acc Date: 26/03/2025 07:45	Report Date: 26/03/25 13:27
Liver Function Test (LFT)			
Total Bilirubin	2.41 ▲ (H)	mg/dl	0 - 1.2
Method - Serum, Colorimetric Diazo Method			
Direct Bilirubin	1.08 ▲ (H)	mg/dl	0 - 0.3
Method - Serum, Diazo Method			
Indirect Bilirubin	1.33 ▲ (H)	mg/dl	0.2 - 0.9
Method - Calculated			
Alkaline Phosphatase	131 ▲ (H)	U/L	40 - 129
Method - Serum, IFCC Colorimetric (PNP)			
Gamma GT	21	U/L	8 - 61
Method - Serum IFCC Enzymatic Colorimetric			
Albumin	2.92 ▼ (L)	g/dl	3.97 - 4.94
Method - Serum Colorimetric with BCG			
Globulin	3.49	g/dl	2.3 - 3.5
Method - Calculated			
A:G Ratio	0.84 ▼ (L)	:1	1 - 2
Method - Calculated			
Total Protein	6.41	g/dl	6.4 - 8.3
Method - Serum, Biuret complex/Colorimetric			
SGPT/ALT	24.3	J/L	< 45
Method - IFCC, Without Pyridoxal Phosphate			
SGOT/AST	40 ▲ (H)	U/L	< 35
Method - IFCC, Without Pyridoxal Phosphate			

Bilirubin Total
Expected Values

Cord Blood < 2 mg/dl

Newborns, Premature :

0-1 day 1.0-8.0 mg/dl

1-2 day 6.0 - 12.0 mg/dl

3-5 day 10.0 - 14.0 mg/dl

Print DateTime: 27/04/2025



LABORATORY INVESTIGATION REPORT

Patient	██████████	Order Date	██████████
UHId	: AHDW.325041	IP NO	: 127101
Episode	: IP	Facility	: Aakash Healthcare Delhi
Ref. Doctor	: Dr. Gastro Unit (Dr. Sharad / Dr. Ekant)		

L,argininemia, ornithinemia), with increased protein intake (hyperammonemia Type II, citrullinuria, arginosuccinic aciduria, lysine intolerance, hyperlysinuria, dibasic aminoaciduria Type II). Blood ammonia measurements are useful in monitoring patients on hyperalimentation therapy. Hyperammonemia may appear in cases of liver malfunction or renal failure. In cases of liver disease, ammonia values rarely surpass 500 $\mu\text{mol/L}$. Significant hyperammonemia during childhood may be observed in a defective ammonia cycle, organic acidemias, transient neonatal hyperammonemia (THAN), and defective oxidation of fatty acids. In such cases, ammonia levels may often surpass 1000 $\mu\text{mol/L}$. Some organic acidemias and ??? may present through symptomatic hyperammonemia from the first day of life. The presence of metabolic acidosis and urine organic acids may assist the distinction between organic acidosis and defective urea cycle. Hyperammonemia may also appear in older infants with defective oxidation of fatty acids. Slight hyperammonemia is relatively common in neonates and can reach values double the normal ones, as it is asymptomatic.

Sample No: 0711203381 Collection Date: 25/03/25 04:54 Ack Date: 26/03/2025 07:45 Report Date: 26/03/25 10:27

Renal Function Test (KFT)

Urea	50.2	mg/dl	18 - 55
Method - Urease-GLDH			
Creatinine	1.74 ▲ (H)	mg/dl	0.67 - 1.17
Method - Serum, Enzymatic			
Uric Acid	6.2	mg/dl	3.5 - 7.2
Method - Uricase-POD			
Phosphorus	3.36	mg/dl	2.5 - 4.5
Method - UV Molybdate			
Calcium	8.51 ▼ (L)	mg/dl	8.8 - 10.2
Method - Serum, BAPTA			
Bicarbonate	22.2	mmol/L	22 - 29
Method - Enzymatic PEP-c			
Sodium	136	mmol/L	136 - 145
Method - Serum Indirect ISE			
Potassium	4.46	mmol/L	3.5 - 5.1
Method - Serum, ISE Indirect			
Chloride	105.4	mmol/L	98 - 107
Method - Serum, Indirect ISE			

Print Date/Time: ██████████



LABORATORY INVESTIGATION REPORT

Patient: [REDACTED] [REDACTED] [REDACTED]
UHId: [REDACTED]
Episode: OP
Ref. Doctor: Dr. PAC (Pre Anaesthesia Checkup)
Facility: Aakash Healthcare Delhi

End of Report



Dr. Anuradha Kumar, MBBS, MD
Pathology

Associate Consultant Pathologist

RegNo: 44795



Dr. Ankush Singhal, MBBS, MD
(Bio-Chemistry)

Associate Consultant & Incharge
Administration

RegNo: 60140

Print DateTime: [REDACTED]





Laboratory Investigation Report

Patient [redacted] Centre [redacted]
[redacted] : [redacted]
[redacted] : Dr. Meenakshi Jain
Collection Date/Time : 27/Apr/2025 10:30AM
Reporting Date/Time : 27/Apr/2025 12:58PM

Clinical Biochemistry

Test		Bio Ref Interval
Urine		
Urinary Potassium (Spot)	10.75	mmol/l
ISE Indirect		

Comment Ref Ranges :-

Male

< 40 years: 11 - 80

≥ 40 years: 17 - 99

Female

< 40 years: 17 - 145

≥ 40 years: 22 - 164

Kindly correlate with clinical findings

*** End Of Report ***

Preeti Tuli

Dr. Preeti Tuli, M.D.
Associate Director & Quality

Meenakshi

Results follow:

Test Performed at : 969 - Max Lab R G Square Mall, Patparganj, 108A, IP Ext, I.P.Extension, Patparganj, Delhi, 11
Booking Centre : 5574 - Max Hospital, Patparganj, 3rd Floor, RG Mall, IP Extension, Patparganj Delhi.
The authenticity of the report can be verified by scanning the Q R Code on top of the





R.k & Narender Parkash

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LAB REPORT

ar,

Patient

Type : SERUM

By : Dr. DEEPAK BHAMBE

Reporting Time : 26-Apr-2025 12:23PM

Test

SEROLOGY

C-REACTIVE PROTEIN(QUANTITATIVE

95.00

mg/l

0.0 - 6.0

C-reactive protein (CRP) is serum protein synthesized in the liver. CRP level increases within hours of an acute injury or the onset of inflammation in conditions like septicemia, meningitis, inflammatory disorders, post surgery or myocardial infarction. It is a semi quantitative test based on the principle agglutination.

WIDAL TEST (Salmonella Antigen) TES

	1/20	1/40	1/80	1/160	1/320
Salmonella Typhi 'O'	+	+	+	+	-
Salmonella Typhi 'H'	+	+	+	+	-
Salmonella Paratyphi 'AH'	+	+	-	-	-
Salmonella Paratyphi 'BH'	+	+	-	-	-

RESULT: WIDAL TEST IS POSITIVE.

In typhoid fever, the H titre is elevated earlier and more frequently than the O titre. Antibody rise is maximal during the second week of illness. Antibiotic treatment did not affect the rise of antibody titre in typhoid fever. A single Widal test in an unvaccinated patient showing H and/or O titres greater than or equal to 1:160 and typhoid-like symptoms is strongly suggestive of typhoid fever. It also appears that H titre is more useful than O titre. More false positives are found than false negatives. Under these circumstances, the clinical picture is the decisive factor in making a diagnosis of typhoid. PMID: 7342378 [PubMed - indexed for M With the cut off value of anti TO = 1:80 and anti TH? = 1:160 Widal test had relatively good NPV (98.9%), but PPV was very low (5.7%). Positive predictive value is more important than other measure of clinical diagnostic methods because it gives the proportion of patients with positive test results that are correctly diagnosed but it is highly affected by a prevalence of the disease. In this study only 7 (2.5%) had culture proven febrile typhoid fever. So a negative Widal test result has a good predictive value for the absence of the disease but a positive result would have a low predictive value for the presence of typhoid fever [9].

TYPHI DOT

TYPHIDOT IgM

POSITIVE

NEGATIVE

TYPHI DOT IgG

NEGATIVE

NEGATIVE

INTERPRETATION :-

Igm positive means acute typhoid fever. IgM& IgG positive means acute typhoid fever in Middle positive means relapse or reinfection or previous infection. IgM&IgG negative means probably typhoid. Rarely high IgM concentration may give false negative For IgM because of specific IgM correlated clinically and with Other laboratory investigation.

Checked by Mohan Kishor

Mohan Kishore
DML IIMT
Sr. Lab Technician



Dr. Amir Razi Khan
M.B.B.S. MD (Path)
Consultant Pathologist
DMC/R/7698



TEJAS HOSPITAL

A Unit of RINSE Marketing Pvt. Ltd.

ISO 9001:2015

A Multispeciality & Gastro Care Center

PATHOLOGY

PATIENT [REDACTED]
[REDACTED]
[REDACTED] : TEJAS HOSPITAL

[REDACTED]
[REDACTED]
[REDACTED] DATE : [REDACTED]

INVESTIGATIONS	VALUE	Unit	REF.RANGE
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BIOCHEMISTRY

KFT(KIDNEY FUNCTION TEST)

BLOOD UREA	↑ 49.8	mg/dl	11-40
BLOOD UREA NITROGEN(BUN)	↑ 23.26	mg/dl	5-20
SERUM CREATININE	1.0	mg/dl	0.7-1.4
SERUM URIC ACID	↓ 2.5	mg/dl	3.2-7.0
SERUM ELECTROLYTE			
SODIUM(Na)+	↓ 124	mEq/L	135-155
Potassium (K)	↓ 2.7	mEq/L	3.5-5.5
CHLORIDE(Cl)+	↓ 94	mEq/L	98-108

Lab Technician

DR VIDYUT PRAKASH
MD(Micro Biology)
Regn No-40221



UNIQUE PATH LAB

(A Unit of Apex Hospital)

: Mob.: [REDACTED]

LAB REPORT



Lab Reg. No. : [REDACTED]
Patient : [REDACTED]
[REDACTED] : Mark Hospital
[REDACTED] : LAB
B.Reg. No. : 394/25

Date / Time : [REDACTED]
Collection Time : [REDACTED]
Reporting Date Time : [REDACTED]

Investigation	Value	Units	Biological Ref Interval
CRYSTALS	None Seen		None Seen
CASTS	None Seen		None Seen
OTHERS	Nil		Nil

Immuno Biochemistry

CRP (C Reactive Protein) Quantitative
20.6 High

mg/L

0.0 - 6.0

CRP QUANTITATIVE
Turbidimetric

A C-reactive protein (CRP), a type of acute phase protein, is used to identify inflammation or infection in the body. CRP is an early indicator of these problems and its levels can rise quickly.

Viral Marker(Hiv,Hcv,Hbsag)

HIV (AIDS) TRI-DOT I & II
Card method
Rapid Card HBSAg
card method
HCV
card method

Negative
Negative
Negative

Negative
Negative
Negative

Serology

Authenticated By

Dr. SAVITA BHARTI
MBBS, DCP

DR. Savita Bharti
MBBS DCP No. 49058
Consultant Pathology
Reg. No. : 49058

★ Clinic Correlation is Necessary for the Final Diagnosis
if Result are Unexpected, please [REDACTED]

Home Collection Facility Available
Not For Medico Legal Purpose



RAINBOW DIAGNOSTIC CENTER

A Unit of Noesis Kronos Pvt. Ltd.

Name : Mrs MANJU DEVI

Reg. No. :RHR139802

IPD/OPD

Accession No. :RL250426083

:ESIC

Dr. : MEDICINE UNIT (DR LN
YADAV, DR PAWAN KUMAR)

IPD/Bill No :OB25007466

Location/Bed.No

Report Gen at: 1:49:50 PM



Accession No

CLINICAL PATHOLOGY



Registration No

SAMPLE TYPE : URINE.

URINE EXAMINATION REPORT

Investigations	Status	Result	Unit	Biological Reference Interval
URINE pH		6.0		5-8
SP. GRAVITY		1.015		1.000-1.030
URINE PROTEIN		TRACE	mg%	
URINE SUGAR		NIL		
URINE KETONES		NIL		
URINE BILIRUBIN		NIL		
URINE UROBILINOGEN		NIL		
URINE PUS CELLS		8-10		0-5
URINE RBC		0-1		0-0
EPITHELIAL CELLS		2-3	/HPF	0-5
BACTERIA		ABSENT		

*** End of Report ***

SHASHI YADAV
Lab Technician



Medical Superintendent
Rainbow Multi-Specialty Hospital
& Trauma Centre
Narnaul, Haryana

Dr. PAWAN KUMAR
MBBS, MD, FN-24518
Physician & Intensivist
Rainbow Multisp. Hospital
Trauma Centre, Narnaul

RIA YADAV
MBBS, MD (PATHOLOGY)
REGD. NO-27490

Lab Technician

DISCHARGE SUMMARY

[REDACTED] : 176203
[REDACTED] : 10:46 AM
Ward/Bed No : B-Block-FOURTH FLOOR / 455-2
Consultant : Dr DR RAVI BANSAL/DR ARUN KUMAR
[REDACTED] : 25/169645
D O D : 20/04/2025
Company : Vidal Health Insurance TPA
Private Limited

CLINICAL PATHOLOGY: URINE R/E, ROUTINE & MICROSCOPY: COLOUR: Pale Yellow, SPECIFIC GRAVITY: 1.015, pH: 6.0, BILIRUBIN: NIL, BLOOD: 2+, PROTEIN: 2+, UROBILINOGEN: NORMAL, PUS CELLS: 2-4, CRYSTALS: NIL,

Date - [REDACTED]

BIOCHEMISTRY: ELECTROLYTES NA, K, CL: SODIUM: 130 mEq/L, POTASSIUM: 4.6 mEq/L, CHLORIDE: 100 mEq/L,

BIOCHEMISTRY: AMMONIA*: AMMONIA: 57.3 μ mol/L,

HAEMATOLOGY: TLC: TLC (Total Leucocyte Count): $7.39 \times 10^3/\mu$ L,

Date - [REDACTED]

BIOCHEMISTRY: CREATININE: CREATININE: 4.60 mg/dL,

BIOCHEMISTRY: ELECTROLYTES NA, K, CL: SODIUM: 130 mEq/L, POTASSIUM: 4.7 mEq/L, CHLORIDE: 99 mEq/L,

HAEMATOLOGY: PROTHROMBIN TIME P.T.*: Patient Value: 12.9 Sec, Mean Normal Prothrombin Time (MNPT): 11.0 Sec, Prothrombin Ratio (PR): 1.17, International Normalized Ratio (INR): 1.18,

HAEMATOLOGY: HAEMOGRAM: HB: 8.2 g/dL, HEMATOCRIT: 25.30 %, RDW-CV: 15.6 %, TLC: $3.89 \times 10^3/\mu$ L, PLATELET COUNT: $114.0 \times 10^3/\mu$ L, ESR: 55 mm / 1 hr, Neutrophil: $2.65 \times 10^3/\mu$ L, Lymphocyte: $0.54 \times 10^3/\mu$ L, Monocyte: $0.58 \times 10^3/\mu$ L, Eosinophil: $0.09 \times 10^3/\mu$ L, Basophils: $0.03 \times 10^3/\mu$ L,

Date: [REDACTED] **X-RAY CHEST AP VIEW (BEDSIDE)** Mild pulmonary venous congestion is noted. Non-homogeneous haziness is seen in both lower zones. Rest of the visualized lung parenchyma does not show any obvious abnormality Both CP angles appear clear. Mild cardiomegaly is seen.



Press Enclave Marg, Sheikh Sarai Phase-II, New Delhi-110017. Ph: 864646417, 011-61426142

Email: info@psri.net URL: www.psrihospital.com
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PATHOLOGY

Patient [REDACTED]
 UHID 1992 Sample Date 27/04/2025 7:50AM
 [REDACTED] [REDACTED] [REDACTED] 8:00AM
 [REDACTED] OPI [REDACTED]
 Referred By Report Date 27/04/2025 8:48AM
 Report Status Final

Test Name	Res	Unit	Biological Ref. Range	Method
Blood Gas Value				
pH	7.3		7.350 - 7.450	
pCo2	41.	mmHg	35 - 45	
pO2	30.	mmHg	80 - 110	
Oximetry Values				
ctHb	12.	g/dL	12.0 - 16.0	
sO2	50.	%	95.0 - 99.0	
Hctc	39.	%		
Electrolyte Values				
cK+	4.2	mmol/L	3.5 - 4.5	
cNa+	131	mmol/L	135.0 - 145.0	
cCa2+	1.0	mmol/L	1.15 - 1.29	
cCl -	103	mmol/L	96.0 - 106.0	
Metabolite Value				
cGlu	135	mg/dL	80.0 - 120.0	
cLac	2.0	mmol/L	0.5 - 1.5	
Acid base Status				
cBase (Ecf) c	-3.	mmol/L	.	
cHCO3-(P,st) c	20.	nmol/L	-	

End Of Report

Provisional Result Entered By :
 SHAREEF KHAN AHC1180

Dr. PRIYANKA MONDAL
 MBBS, MD (PATHOLOGY),
 Reg No.: DMC 115146



MAHINDRU PATHOLOGY LAB

(FULLY COMPUTERISED 24 HOURS LABORATORY)

E-1 KIRAN GARDEN, MAIN NAJAFGARH ROAD, UTTAM NAGAR, NEW DELHI-110059

Tel.: [REDACTED] 41752913, 41046417, 40586579 Extn. 516

Date [REDACTED]

UHID.No [REDACTED]

Sample Coll. [REDACTED] 18:48:00

[REDACTED]

MAHINDRU HOSPITAL (IPD)

Sample Type Blood

Female

Test [REDACTED]

W.B.Cs:Show increased total count with neutrophilia.

Platlets:Normal in number and morphology.

No immature cell of any series seen.No haemoparasite seen.

**IMPRESSION :- NORMOCYTIC NORMOCHROMIC WITH
NEUTROPHILIC LEUCOCYTOSIS.**

C-REACTIVE PROTEIN (QUANTITATIVE)

0.2

0.1 - 6.0

****End Of Report****

TECH. [REDACTED]

[REDACTED] KUMAR

SR. CONSULTANT (PATHOLOGIST)

• Clinical correlation is essential for final diagnosis • If test results are unsatisfactory please

• This report is for perusal of doctors only • All disputes are subject to Delhi jurisdiction

for legal

Mr ANANTHRAJ S ,

75 Y/Male

Referred by:

Dr SENTHILKUMAR R MD,PDCC

Requested on : / 16:4
Specimen Collected on : / 16:47
Reported on : / 16:47



TEST NAME	RESULT VALUE	BIOLOGICAL REFERENCE RANGE
BIOCHEMISTRY		
ELECTROLYTES		
SODIUM - SERUM	136	130 - 148 mEq/L
POTASSIUM SERUM	5.0	3.5 - 5 mEq/L
BICARBONATE SERUM	20	22 - 26 mEq/L
CHLORIDE - SERUM	107	96 - 108 mEq/L

-----End of Report-----

RESULT ENTERED BY : LABV1

RESULT VERIFIED BY : LABV1

FINAL REPORT



Laboratory Investigation Report

Patient		Centre	
		Collection Date/Time	: 26/Apr/2025 06:46PM
	: Dr.Vyom Aggarwal	Reporting Date/Time	: 26/Apr/2025 08:32PM

Hematology



SIN No:SB2106884

Pooja

Dr. Pooja Bhasin M.D.
Director & HOD
Lab Service Pathology

Vijay

Dr. Vijay Laxmi Sharma, MD
Associate Director & Quality

Test Performed at :1060 - Max Hospital Shalimar Bagh, Max Lab

Booking Centre :1060 - Max Hospital Shalimar Bagh, Max Lab,

The authenticity of the report can be verified by scanning the Q R Code on top of the



MC-2262



SITARAM BHARTIA
Institute of Science & Research

LABORATORY INVESTIGATION REPORT

Patient	██████████	██████████	██████████	██████████
UHId	: SBID.325941	██████████	:	██████████
Episode	: IP	██████████	:	1515162 /
Ref. Doctor	Dr. Medicine Unit /Pradeep Narayan Chauhan	██████████	██████████	██████████
██████████	██████████	██████████	██████████	██████████

-The specimen submitted is processed for culture by conventional method on standard sterile culture media and incubated aerobically at 37°C for up to 48 hours.

-If any significant growth of pathogenic bacteria is indicated during the period of incubation, the isolated organism is further processed for identification by automated VITEK-2 ID & AST system /standard conventional biochemical tests and antimicrobial sensitivity testing is done by automated VITEK-2 ID & AST system/ Kirby Bauer disk diffusion method.

-The testing & interpretation of antimicrobial sensitivity is done as per the latest CLSI/EUCAST guidelines and reported accordingly.

Note- Administration of antibiotic prior to specimen collection may inhibit the growth of pathogenic bacteria.

All the culture reports should always be correlated with relevant clinical & laboratory findings before consideration of treatment.

End of Report

Dr.Monika Singh ,
Consultant - Microbiology
RegNo: 35786

Print DateTime: ██████████





SITARAM BHARTIA
Institute of Science & Research

LABORATORY INVESTIGATION REPORT

Patient [REDACTED]
UHId : SBID.325941
Episode : IP
Ref. Doctor : Dr. Medicine Unit /Pradeep Narayan Chauhan
[REDACTED]

[REDACTED]
[REDACTED] : [REDACTED]
IP NO : 1515162
[REDACTED]
[REDACTED]

Biochemistry

TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Sample No [REDACTED]	Collection Date [REDACTED] 09:51	Ack Date [REDACTED] 10:02	Report Date [REDACTED] 12:37

Procalcitonin -PCT

Sample Type- Serum

Procalcitonin -PCT < 0.05 ng/ml 0 - 0.05

Method - Fluorescence Immunoassay

Interpretation:-

<0.05ng/ml - Normal Value

0.05 - 0.49ng/ml - Minor or no significant systemic inflammatory response. Local inflammation and local infection are possible

0.5 - 1.9 ng/ml - Significant but moderate systemic inflammatory response. Infection is possible, but various conditions are known to induce PCT as well (Severe trauma, major surgery,cardiogenic shock)

2.0 - 9.9 ng/ml - Severe systemic inflammatory response. Most likely due to infection (sepsis) unless other causes are known (see above)
High risk for developing organ dysfunction.

=/>10.0 ng/ml Important systemic inflammatory response almost exclusively due to severe bacterial sepsis or septic shock. Frequently associated with organ dysfunction.

Clinical correlation/ confirmation with other method is recommended)

Print DateTime: [REDACTED]





SITARAM BHARTIA
Institute of Science & Research

LABORATORY INVESTIGATION REPORT

Patient	██████████	██████████	██████████	██████████
UHId	: SBID.325941	██████████	:	██████████
Episode	: IP	██████████	:	1515162 /
Ref. Doctor	Dr. Medicine Unit /Pradeep Narayan Chauhan	██████████	██████████	██████████
██████████	██████████	██████████	██████████	██████████

-The specimen submitted is processed for culture by conventional method on standard sterile culture media and incubated aerobically at 37°C for up to 48 hours.

-If any significant growth of pathogenic bacteria is indicated during the period of incubation, the isolated organism is further processed for identification by automated VITEK-2 ID & AST system /standard conventional biochemical tests and antimicrobial sensitivity testing is done by automated VITEK-2 ID & AST system/ Kirby Bauer disk diffusion method.

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Note- Administration of antibiotic prior to specimen collection may inhibit the growth of pathogenic bacteria.

All the culture reports should always be correlated with relevant clinical & laboratory findings before consideration of treatment.

End of Report

Dr.Monika Singh ,
Consultant - Microbiology
RegNo: 35786

Print DateTime: ██████████





SITARAM BHARTIA
Institute of Science & Research

LABORATORY INVESTIGATION REPORT

Patient [REDACTED]
UHId : SBID.325941
Episode : IP
Ref. Doctor : Dr. Medicine Unit /Pradeep Narayan Chauhan

[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

Biochemistry

TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Sample No [REDACTED]	Collection Date [REDACTED] 09:51	Ack Date [REDACTED] 10:02	Report Date [REDACTED] 12:37

HbA1c

Sample Type- Whole blood EDTA

HbA1c **6.1 ▲ (H)** % < 5.7

Method - HPLC

Result **A1C**

Normal less than 5.7 %

Prediabetes 5.7 % to 6.4 %

Diabetes 6.5 % or higher

Interpretations:

1. HbA1C has been endorsed by clinical groups and American Diabetes Association guidelines 2017 for diagnosing diabetes using a cut off point of 6.5%
2. Low glycated haemoglobin in a non diabetic individual is often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency and haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
3. In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control.

Excellent control-6-7 %

Fair to Good control – 7-8 %

Unsatisfactory control – 8 to 10 %

Poor Control – More than 10 %

Clinical Pathology

TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Sample No 07E0095085	Collection Date [REDACTED] 14:57	Ack Date [REDACTED] 15:03	Report Date [REDACTED] 15:37

Urine RE and ME (Routine and Microscopic)

Sample Type- Urine

Gross Examination

Colour Pale Yellow Pale Yellow

Method - Visual/Gross examination

Print DateTime: [REDACTED]





R.k & Narender Parkash

MULTISPECIALITY HOSPITAL

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LAB REPORT

ar,

Patient

Type : SERUM

By : Dr. DEEPAK BHAMBE

Reporting Time : 26-Apr-2025 12:23PM

Test

SEROLOGY

C-REACTIVE PROTEIN(QUANTITATIVE

95.00

mg/l

0.0 - 6.0

C-reactive protein (CRP) is serum protein synthesized in the liver. CRP level increases within hours of an acute injury or the onset of inflammation in conditions like septicemia, meningitis, inflammatory disorders, post surgery or myocardial infarction. It is a semi quantitative test based on the principle agglutination.

WIDAL TEST (Salmonella Antigen) TES

	1/20	1/40	1/80	1/160	1/320
Salmonella Typhi 'O'	+	+	+	+	-
Salmonella Typhi 'H'	+	+	+	+	-
Salmonella Paratyphi 'AH'	+	+	-	-	-
Salmonella Paratyphi 'BH'	+	+	-	-	-

RESULT: WIDAL TEST IS POSITIVE.

In typhoid fever, the H titre is elevated earlier and more frequently than the O titre. Antibody rise is maximal during the second week of illness. Antibiotic treatment did not affect the rise of antibody titre in typhoid fever. A single Widal test in an unvaccinated patient showing H and/or O titres greater than or equal to 1:160 and typhoid-like symptoms is strongly suggestive of typhoid fever. It also appears that H titre is more useful than O titre. More false positives are found than false negatives. Under these circumstances, the clinical picture is the decisive factor in making a diagnosis of typhoid. PMID: 7342378 [PubMed - indexed for M With the cut off value of anti TO = 1:80 and anti TH = 1:160 Widal test had relatively good NPV (98.9%), but PPV was very low (5.7%). Positive predictive value is more important than other measure of clinical diagnostic methods because it gives the proportion of patients with positive test results that are correctly diagnosed but it is highly affected by a prevalence of the disease. In this study only 7 (2.5%) had culture proven febrile typhoid fever. So a negative Widal test result has a good predictive value for the absence of the disease but a positive result would have a low predictive value for the presence of typhoid fever [9].

TYPHI DOT

TYPHIDOT IgM

POSITIVE

NEGATIVE

TYPHI DOT IgG

NEGATIVE

NEGATIVE

INTERPRETATION :-

IgM positive means acute typhoid fever. IgM& IgG positive means acute typhoid fever in Middle positive means relapse or reinfection or previous infection. IgM&IgG negative means probably typhoid. Rarely high IgM concentration may give false negative For IgM because of specific IgM correlated clinically and with Other laboratory investigation.

Checked by Mohan Kishor

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TEJAS HOSPITAL

A Unit of RINSE Marketing Pvt. Ltd.

ISO 9001:2015

A Multispeciality & Gastro Care Center

PATHOLOGY

PATIENT [REDACTED]
[REDACTED]
[REDACTED] : TEJAS HOSPITAL

[REDACTED]
[REDACTED]
[REDACTED] DATE : [REDACTED]

INVESTIGATIONS	VALUE	Unit	REF.RANGE
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BIOCHEMISTRY

KFT(KIDNEY FUNCTION TEST)

BLOOD UREA	↑ 49.8	mg/dl	11-40
BLOOD UREA NITROGEN(BUN)	↑ 23.26	mg/dl	5-20
SERUM CREATININE	1.0	mg/dl	0.7-1.4
SERUM URIC ACID	↓ 2.5	mg/dl	3.2-7.0
SERUM ELECTROLYTE			
SODIUM(Na)+	↓ 124	mEq/L	135-155
Potassium (K)	↓ 2.7	mEq/L	3.5-5.5
CHLORIDE(Cl)+	↓ 94	mEq/L	98-108

Lab Technician

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